2 Venepuncture

HME922 Procedural Station · 10 minutes · Venepuncture guideline + DOPS form

guideline wording — say it aloud DOPS On the examiner's DOPS marking form ☐ tick when done

KEY MESSAGE

Feel for a soft, bouncy vein. Tourniquet off within **2 minutes**. Follow the **order of draw**. Max **2 attempts**. Label tubes **at the bedside**.

UNIVERSAL OPENING

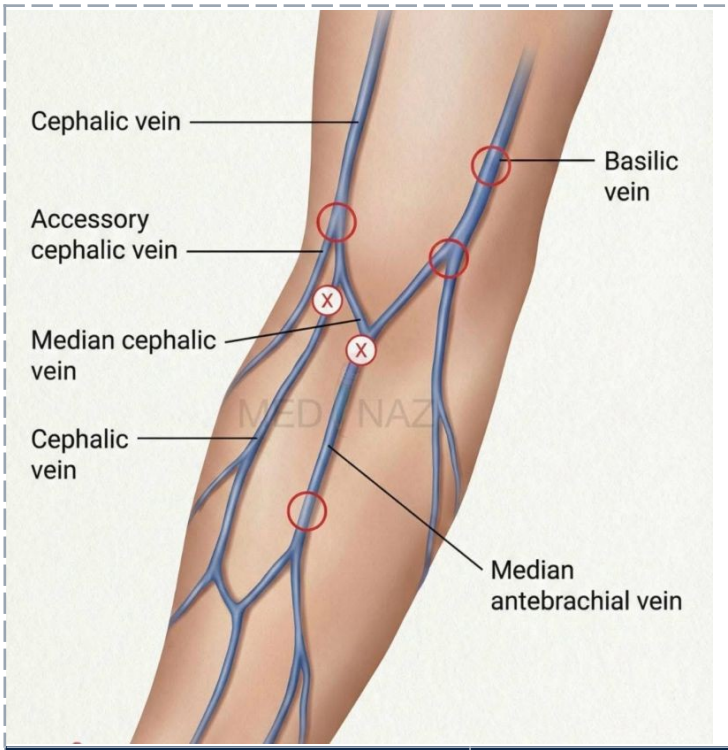
- ☐ Hand hygiene — Moment 1 DOPS
- ☐ Introduce yourself: name + "medical student"
- ☐ Confirm ID — **3 identifiers**; patient states name + DOB; check band against request slip DOPS
- ☐ Explain procedure + which tests
- ☐ Ask about **allergies** (latex, tape, alcohol) and **fainting history** DOPS
- ☐ Allow questions DOPS
- ☐ Gain **informed verbal consent** DOPS
- ☐ Privacy, lighting, exposure; seated/reclined, arm at or below heart DOPS

MEMORY AID — 5 MOMENTS

5 Moments of Hand Hygiene: 1 before touching patient · 2 before procedure · 3 after body-fluid risk · 4 after touching patient · 5 after touching surroundings

CLINICAL KNOWLEDGE — SAY IT

- ☐ Venepuncture = sample for lab tests. Venesection = therapeutic removal (haemochromatosis, polycythaemia)
- ☐ **Sites:** **cephalic, median cubital, basilic** — superficial, small nerve supply. Dorsum of hand = less prominent, more painful
- ☐ **Avoid:** lower limbs, small/non-palpable, repeatedly used, bruised/inflamed/sclerosed, infection, broken skin, cellulitis, phlebitis, lymphangitis, **arm with IV infusion**, limb for vascular surgery, **mastectomy side**, **AV fistula**; caution coagulopathy



Complication	Prevent/manage

Syncope	Fainting history → lie supine
Haematoma	Loosen tourniquet, remove needle, **firm pressure**
Infection	Uncommon — aseptic technique
Needlestick	Sharps bin at bedside, immediate disposal
Haemolysis	Falsely high **K+**; usually technique
Not enough blood	Paediatric tubes if difficult
Prolonged tourniquet	Falsely ↑ protein, AST, lipids, cholesterol, iron; affects PCV

PREPARE

- ☐ Check request slip for tubes; enough kit for **two attempts**
- ☐ Gloves (non-sterile, latex free), eye protection, sharps bin, kidney dish, **Vacutainer** (preferred closed system) / butterfly, alcohol swab, tourniquet, bluey, tubes, cotton ball + tape, labelled slip, specimen bag DOPS
- ☐ Difficult vein: **21 G butterfly**, or 21 G + 10–20 mL syringe
- ☐ Blood alcohol test → **only the kit swab**, no alcohol swab
- ☐ Same arm as IV fluid (no choice): stop infusion, wait **5–10 min**, sample distal to cannula

PROCEDURE

1. Tourniquet **10–15 cm above** site — fingers fit under it; arterial flow not blocked DOPS
2. Palpate (before gloves): soft, bouncy, refills, doesn't roll. **Pulsates = artery → don't use** DOPS
3. No vein? Fist clench/unclench; warm pack or warm water 5–10 min
4. Gloves; alcohol swab; **dry 30 seconds**; don't re-touch DOPS
5. Anchor vein — thumb below site, skin taut DOPS
6. Needle **bevel up, 10–30°**, in line with vein DOPS
7. Push tubes in, fill until vacuum stops; **gently invert** additive tubes DOPS
8. Tourniquet released by **2 minutes**
9. Tourniquet **fully off** → cotton ball → withdraw needle → firm pressure **1–2 min**
10. Bending the elbow is **NOT** pressure → haematoma
11. All tubes labelled **at the bedside**

> **MEMORY AID — ORDER OF DRAW — "Be Loving, Respectful, Gentle, Patient, Generous, Brave"**
> **B**lood cultures (sterile) → **L**ight blue (citrate) + ACD → **R**ed / gold (clot activator) → **G**reen (heparin) → **P**urple (EDTA) → **G**rey (glucose) → **B**lack (ESR) → other

TROUBLESHOOTING

Problem	Fix
No flow	Check position, advance slightly
Through the vein	Pull back slightly
Bevel against wall	Adjust angle
Vein collapsed	Tighten tourniquet, gentle fist
Haematoma forming	Release tourniquet, withdraw, firm pressure
2 failed attempts	Ask a senior colleague

UNIVERSAL CLOSING

- ☐ Needle straight into sharps — **never recap**

- ☐ Waste: sharps / biohazard / general DOPS
- ☐ Area clean; patient comfortable; bed lowest; call bell DOPS
- ☐ Check patient isn't light-headed before you leave; report bleeding/swelling
- ☐ Hand hygiene — Moments 4/5 DOPS
- ☐ Request slip dated, timed, signed; specimen bag with slip in outer pocket → lab DOPS
- ☐ Can't answer? "I'll pass that on to someone senior"
- ☐ Examiner asks **"How do you think that went?"** → one thing done well · one to improve · one strategy

MY NOTES